

Monash Medical Centre Adult Emergency Medical Model of Care

Revision, commencing February 2 2026

Aims

Simplify and rationalise medical workforce allocations and patient flows to improve patient care and junior staff supervision

Exclusions from scope

- Changes to nursing model of care
- Changes to staffing numbers

Rationale

Current allocation of patients to medical team and waiting areas is based on infection risk during COVID. This method of allocation is no longer fit for purpose, with other procedures currently in place able to deal appropriately with infection risk issues. Current method of allocation produces clinical risk and lack of task balance between medical practitioner work teams in ED.

Summary of changes

- All patients (apart from exceptions below) are alternatively triaged to either Amber or Green stream (regardless of clinical presentation, including COVID patients).
- Removal of patient care group allocation based on infection risk.
- No adult patient should be allocated to Red stream
- Exceptions
 - All mental health and drug and alcohol patients continue to be allocated to Amber stream.
 - All Clinic suitable patients should be triaged to Clinic.

Waiting Room allocation

- Waiting Room North
 - All Cat 2 patients, AV offload patients, and any others in judgement of triage nurses with SMS consultation if needed
 - After RAT, low clinical risk Cat 2 patients can be moved to Main Waiting Room
- Main Waiting Room
 - Other Amber and Green patients
 - Clinic patients if Clinic Waiting Room is full
 - Cat 2 patients if classified as low risk after medical assessment
- Clinic Waiting Room
 - Clinic patients as per current practice, until full

Senior Staff Roles

- Green consultant
 - Oversight of Green patients
 - Admitting officer phone
- Amber consultant
 - Oversight of Amber patients
 - Oversight of Hub
- Clinic consultant/Reg
 - Oversight of Clinic patients
- Resus consultant/Reg
 - Oversight of resus
 - Oversight of AV corridor
 - Review of all AV Cat 2 patients if possible
- SSU consultant
 - Oversight of SSU

Allocation of JMS to medical teams

- Four team model (Morning and afternoon) : Green, Amber, Clinic, SSU
 - Principles:
 - Registrars see in time order across Green, Amber, Clinic
 - Interns/HMOs assigned to a specific team
 - Extra Clinic doctor will be assigned when waiting patients in clinic exceeds 8 or if waiting time in clinic exceed 3 hours.
- Two team model overnight (Mains and SSU)

New model JMS staff allocation

- Morning
 - Registrars see in time order across Green, Amber, Clinic
 - HMOs and interns are assigned to green or amber
 - One HMO in Hub
- Afternoon
 - Registrars see in time order across Green, Amber, Clinic
 - HMOs and interns are assigned to green or amber
 - One Registrar in Hub
 - Dedicated Clinic HMO when staffing allows
- SSU allocation of HMO/Intern unchanged

Changing patient care group allocation can only be done by discussion between relevant SMS team leaders